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OVERVIEW:

Company Summary

CORPORATE PARTICIPANTS

Eliav Barr Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Peter Dannenbaum Merck & Co Inc - Senior Vice President, Investor Relations

CONFERENCE CALL PARTICIPANTS

Umer Raffat EVERCORE ISI - Analyst

PRESENTATION

Umer Raffat - EVERCORE ISI - Analyst

Okay. Excellent. Thank you, guys for joining us. Super excited to have Merck management with us. Just before I dig into my specific questions, I will let you frame the broader discussion. We'll jump right in.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Well, great. It's great to be here. My name is Eliav Barr. I'm Chief Medical Officer at Merck. It is a tremendous period of time for us.

We have probably some of the most exciting pipeline products that I've had in my 30 years in the company and across oncology, vaccines and ID and general and specialty medicines. So I really look forward to talking to all of you.

QUESTIONS AND ANSWERS

Umer Raffat - EVERCORE ISI - Analyst

Excellent. So I realize this is going to be a very R&D-focused discussion, and that's the direction I want to go. But just ahead of that, Peter, could you just refresh us on where we stand on the broader financials, especially as it relates to the GARDASIL situation? And are we in all clear now after the third quarter (inaudible)?

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

We are definitely -- we've had a great year at Merck. Our current guidance is above where we initiated guidance at the beginning of the year. We expect 9% to 10% ex-FX revenue growth on a full company basis.

GARDASIL, we expect to be roughly flat year-over-year because there's been a slowdown in China. But the rest of the business has been extremely strong. And GARDASIL has been extremely strong in markets outside of China. So in the third quarter GARDASIL grew strong double digits in just about every major region around the world. And we're in a very good position of understanding what's going on in China.

And we are addressing that with our partners Zhifei. And as we look out to 2025, we believe we will have a year of solid growth for Merck overall. So we're in a very good position commercially, including with GARDASIL.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Excellent. And maybe just one quick additional one just before we dig into some of the R&D programs. I know there was this I think it's -- I would call it aspiration number put out that oncology would go to \$20 billion pipeline, non-KEYTRUDA pipeline. Oncology is \$20 billion, cardiometabolic, \$15 billion.

And immunology is multibillion. Does that factor in on a risk-adjusted basis on some sort of long-term forecast? Is that something you guys would ever consider doing?

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Yes. So we've outlined what we believe our commercial opportunity is in several parts of our pipeline, not our entire pipeline, but as Umer mentioned, we've laid out an expectation for greater than \$20 billion of commercial opportunity as we approach the mid-2030s from assets in our oncology pipeline, excluding anything that we're doing around KEYTRUDA, including our subcutaneous program, which we just read out of successful Phase 3 on. Those are all on a nonrisk-adjusted basis and we'll determine how we outline our future opportunities in future settings at some other time. But for now, that's -- we've outlined oncology, cardiology, and immunology as three major sources of opportunity. We also have opportunities in other areas of the pipeline, HIV and vaccines, and also in our animal health business.

Umer Raffat - EVERCORE ISI - Analyst

Excellent. So maybe let's start with oncology. That was the first line. That was \$20 billion. And you just clarified that does not include any of KEYTRUDA's new indications and/or KEYTRUDA subcu.

However, does that include your new Chinese PD-1 VEGF, for example?

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

So not specifically.

Umer Raffat - EVERCORE ISI - Analyst

Not at the time it was given.

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Yes, at the time it was given, we had not entered into the transaction with LaNova. So that is a future additional opportunity that would be incremental if it works out clinically.

Umer Raffat - EVERCORE ISI - Analyst

Okay. So maybe let's start with that topic, I remember I hosted you at our offices back in June. This was shortly after ASCO. Obviously, very interesting data from Summit Therapeutics side with the hazard ratio they've put up on ivonescimab. But we'll see how the profile plays out on an OS basis.

A, I'd be curious what are your thoughts on OS evolution and just your broad take on that data set and ask some more specific ones on the program you guys routing?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Sure. Well, I think that the data are very interesting. They were -- they demonstrated a pretty substantial hazard ratio for progression-free survival. And the issue for us and for everyone in the field is that the regulatory endpoint and the endpoint that payers and patients value most is overall survival. And we need to -- we just need to follow that to its conclusion.

We've seen data sets with anti-VEGF drugs where the PFS didn't translate to OS, and we've seen some of that have. So the proof will be in the pudding. We'll see when the data read out and become more mature.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Excellent. So I guess -- and I want to -- I have several follow-ups on this. First one, as you think about the program you guys brought in, is that more an insurance against anything that happens on the PD-1 VEGF space? Or is that just your conviction in the target?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes. I think it's the latter, not the former. -- because by the time the products get into the US setting will be close to the US LOE for KEYTRUDA was really -- that wasn't really a factor to this.

My interest was really more about seeing the data, reflecting back on the large number of studies that have been done -- that we've done with various kinds of VEGF inhibition, whether it's through TKI mechanism or through direct anti-VEGF and asking ourselves if we were to look at the KEYTRUDA database, where would we position this opportunity and how would we prosecute it. And so we thought about it. We looked at the data that were generated by Akeso Summit. And then based on that, looked at the profiles of what we wanted to see in a molecule, we looked at a variety of different drugs or candidates, I should say. And we ended up with the LaNova LM 299 as the one.

Umer Raffat - EVERCORE ISI - Analyst

Got it. As I think about the signals seen to date, hazard ratio profound, it's on a PFS basis. presumably as we go into a KEYTRUDA combination trial, would you a similar PFS hazard ratio to replicate? And because there's a lot of reasons to expect or not expect.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

So we wouldn't -- I mean, we would be in a trial against KEYTRUDA.

Umer Raffat - EVERCORE ISI - Analyst

KEYTRUDA chemo?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

So -- Yes, sorry, yes. So I think that the -- where this will play out will depend again on the efficacy and safety profile. And if you look at the data from -- that were presented -- and there was the data on greater than 1% frontline in NSCLC and there was the 1% to 49% data and the greater than 50% data -- and you saw that the effect size in the greater than 50% data population was much greater. And you had that kind of hockey stick kind of thing for 1% to 49% that always worries when I was worried about with VEGF inhibitor combinations where you had a really big effect that at the beginning and then it kind of back close and one worries about that.

So how we'll develop the drug will depend on what we see in the Phase 1 program. We've got a lot of insights from the KEYTRUDA program that we think will help in managing efficacy, toxicity factors. And I think that, that's going to be a very important thing with these drugs is to ensure that you have the maximum on the efficacy and keep a vigilant eye on safety.

Umer Raffat - EVERCORE ISI - Analyst

Got it. What do you think about Chinese patients responsiveness to -- Chinese patients and their responsiveness to VEGF? And how would that dynamic be relevant or the.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

So a lot of the BEV studies have shown a higher response rate in Chinese patients, but the -- but the data, I think, we're so spectacular. So there was so much of a PFS benefit in the Akeso Summit data that the camera data, I think it's hard to imagine that it's just a --

Umer Raffat - EVERCORE ISI - Analyst

And maybe just the last one on this because we have a lot of topics to get into, but in their EGFR mutant study, PFS is also still very robust, I think it's 0.49, but OS evolves into a hazard ratio that's above 0.8. Do you see a phenomenon like that happen on the -- on the trial that was just reported this past week.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Well, we'll see. I mean, again, I just -- I think that's why you do OS the valuations, you have to make sure that you get the right patients. I mean part of it is also -- we have to remember that there is a second line therapy and third-line therapy adds more and more noise to the system. So you have to be careful about understanding what happened afterwards. But if you look at some of the trials that were done with pembrolizumab and anti-angiogenesis agents, you see this kind of profound initial and then slow down afterwards.

And so that's why we've been -- we're still -- whenever I talk about it internally, externally, when we design our clinical program, we have to keep an eye out on the tail end, not the front.

Umer Raffat - EVERCORE ISI - Analyst

Any questions on this topic just before we move on. Okay. So maybe a couple of additional follow-ups on oncology then. HER3, the lung study. I guess what are your -- I think you guys met the primary endpoint of PFS, but your confidence and OS on that.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It was immature. I can't say. -- happily, we've got a bunch of other further analyses ongoing.

Umer Raffat - EVERCORE ISI - Analyst

Why did Astra pass on this one. That just never made sense.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Why did they pass on this asset?

Umer Raffat - EVERCORE ISI - Analyst

Yes. Because they could have put this as part of the collaboration with Daiichi.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

(inaudible), I don't know. Okay. All right.

Umer Raffat - EVERCORE ISI - Analyst

Excellent.

And -- and on the PFS, is it reasonable to think there's at least a 2-plus month median PFS benefit?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I think that there is -- we haven't shown the data because it was even those data were relatively immature. So we have to wait and see how it goes. I think that the -- what was interesting to me, and if you look at the study results at the study, it was HER3 against chemo induction followed by maintenance. And there were differential effects in the front and the back end.

Umer Raffat - EVERCORE ISI - Analyst

(inaudible) effects in the front and the back end.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

During the induction and the maintenance. So you have to -- that's why we keep on saying we just wanted to see how the data mature.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Got it. Got it. I think there were two grade 5 ILDs as well.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

There was -- yes, these drugs and a lot of the -DXd drugs, which true for across the Daiichi theories, they do have some ILD, but there -- one of the things we've learned, and this is true, I think probably also with the Astra agents is that they have a very strong program to detect written reduce and prevent it from happening. There were two cases, but that it was compared with what we would have expected it was about right.

Umer Raffat - EVERCORE ISI - Analyst

Got it. CDH6, I think this ADC has I think every time I see it in a conference update, the dose has stepped down. I guess, are we now hitting a point where you can get to therapeutic index, which kind of works on both sides, efficacy and safety?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes, I have to say that -- so you worry about the dose, I think about this as one of the coolest of the drugs that we got. Yes. It's because between 4.8 and 6.4 mg/kg 4.8, 6.4 the efficacy is so extraordinary. And the things that I really love is the duration of response and the PFS. It's really, really good.

And so I see this as being a real sleeper in the series. But for me, it's going to be -- we're excited to bring it into now later-stage studies and I'm very confident that it's going to be a big home run.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Excellent. Anything else on oncology just before we transition further .

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Well, we had a lot of preclinical -- there's a lot of preclinical data. There's no -- the clinical studies had just started, and we had maybe two or three patients. So I think that the configuration and the preclinical data and our profiling of that molecule versus others, we thought that was the best kind of stoichiometry between PD-1 inhibition and VEGF inhibition, the right kind of potential half-lives projected half-lives and so on. It's the earliest in .

Umer Raffat - EVERCORE ISI - Analyst

Well, the one on PD-1 and VEGF.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's something.

Umer Raffat - EVERCORE ISI - Analyst

It's something. Okay. That's good to know. Okay. Excellent. And when is the first data -- like, I guess, what will be the cadence of data updates? How fast can you be in a randomized trial is the real question?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's in dose escalation right now. So it will take some time.

Umer Raffat - EVERCORE ISI - Analyst

Got it. And could we envision a scenario where you run a trial like assuming this target is active, which is -- but could you envision a scenario where a KEYNOTE-21G like size trial could enable a path to registration.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I don't know. It's going to be -- it really depends on setting -- we're going to Yes, I know. I mean I just -- I think that it's that era may not necessarily be there anymore. One of the things that I think is important with these drugs is the safety database. And I think that regulatory agencies will want to see not just efficacy and not just a small safety database.

They want to see longer-term follow-up.

They'll want to see the effect size with OS. They'll want to see perhaps the proper dose and having an optimistic like dose. So when you think about all of that 120 patients is not that -- won't get you there. So it depends. We're going to -- one of the things we have to remember that this drug needs to pass the US regulatory gauntlet, which means Project Optimus, which means making sure that they're confident in durability and the long-term efficacy and safety profile. So I'm not sure that it's reasonable to expect some small study to get to a registration precise effect. We have a variety of different drug combinations that we want to try.

Umer Raffat - EVERCORE ISI - Analyst

Okay, got it. Maybe, I guess my last one on this would be -- if you put on a really skeptical hat, you could say, you know what, the entire curve separation happens at the first time point, that's week 6. And you can make a case that the Summit drug is basically a high-dose Keytruda, it's at a higher dose. How do you guys balance those things as you think about the development of.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's a great question, and it's something that we have to address through dose selection and through looking at the effect size for not just that molecule, but also, for example, the one that is the BioNTech combo because those guys are our PD-L1, VEGF. So I don't know. There they had good effect size in triple-negative breast cancer. And so you think about is this just high dose .

Umer Raffat - EVERCORE ISI - Analyst

Can you consider running a high-dose KEYTRUDA trial versus standard of KEYTRUDA in lung? .

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I don't think we would -- I think we optimized the dose of KEYTRUDA. There was a lot of work that was done upfront. I don't think we would do that.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Okay. But are you leaving the door open and that could be a possibility (inaudible) PD-1.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I don't think so. I think it's a combination of PD-1 and VEGF inhibition.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Excellent. Another trial that's important next year, at least to me, is the Group II PH trial. That's the left heart disease trial. But one of the things that worries me is the risk of background anticoagulant us driving a leading imbalance.

Do you have visibility on how the bleeding profile is looking in that trial?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's a blinded study, so I don't have any like by treatment group data. But I have to say that overall, there haven't been too many bleeding events that has not been an issue.

Umer Raffat - EVERCORE ISI - Analyst

Okay. And I know it took a while to recruit and some of the feedback I've picked up is that the trials designed for the trial is designed for pulmonologists, but it's really intended for cardiologists and there's some disconnect some along the way, which is why it's slower to recruit. Any thought process in only taking CPC PH and not the (inaudible).

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes. So we -- so first of all, you're right, these are -- this is -- you're right in the sense that this is not a disease entity that people are warehouse that they have a standard treatment approach that they have the right drugs, and they're just looking for another drug in that category. These are patients who don't have very many options and where the diagnosis needs to be made in a more sophisticated way.

CPC PH kind of patients tend to have a more severe disease. They tend to go down faster. They tend to have more fibrosis. They tend to have more well hypertrophy. The kind of biology that we've seen with PAH with Group one PH.

And so we thought that, that group would be more similar in terms of biology to PAH and that's why we chose it.

I think that the way I look at it is if the study is positive, people will have a rationale for building for going there, and there'll be a lot more diagnosis. I think it's a highly underdiagnosed disease.

Umer Raffat - EVERCORE ISI - Analyst

Peter, would you -- does this have to be repriced (inaudible) if this indication works?

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Yes. We've not spoken to what the pricing strategy would be for WINREVAIR, if this works. So .

Umer Raffat - EVERCORE ISI - Analyst

Okay. Okay. Okay. Makes sense. Oral PCSK9, that's another one that's important for next year.

I think the food and diet restriction topic is coming up more and more and more. Could you just walk us through how significant is that issue or not? And knowing that presumably it will be as part of a cocktail, so there's some TDIs as well. So could you speak to that?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes, the DDI issue is not -- I don't think going to be a problem. We have 30 minutes between administration and food. You can take it with water and sort of non-fatty liquids and then you can have breakfast 30 minutes. This was similar to the bisphosphonates back in the day. And so I think that it's -- I -- it hasn't been a compliance issue in the clinical studies so far.

And obviously, is something that we're going to have to make sure as well people are well educated about. (inaudible) onset is a much more -- much of a problem compared with, let's say, having to get the injections and so on.

Umer Raffat - EVERCORE ISI - Analyst

Got it. I guess the other one is on LDL reduction, there's a CTAP inhibitor in the marketplace as well, which is reporting LDL reduction, which approaches what you guys are about to show. However, I recall Anacetrapib also had LDL reduction akin to the other CTAP inhibitor from New Amsterdam. But the nuance was they were using this LDL measurement method, B which may not be what's the standard. So could you just remind us all the LDL reduction data you guys have shown on an apples-to-apples basis, how different or similar is it versus the new Amsterdam (inaudible).

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

So we use the standard approach to measuring cholesterol, and the reductions that we see with PCSK9 are more profound. And I think that the issues with CETP inhibitors is that they have a pleiotropic effect and the efficacy, the cholesterol reductions have been a little bit variable, and I think there's been some disappointment in some of the numbers. But overall, for us, it's the same measurement system that we use with statins. And throughout our program, we had reductions around 60% above and beyond maximum statins, high intensity statins. So I'm very confident about that.

Umer Raffat - EVERCORE ISI - Analyst

There was one death, I think, 18-meg dose which was possibly drug-related. Would you happen to recall that.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes, it was a motor vehicle accident -- there's someone who crashed their car, with the --

Umer Raffat - EVERCORE ISI - Analyst

How would it be drug related?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

And we never challenge our investigators.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Got it. There was very limited dose response of 12 mg, but you guys on the 20 mg dose anyway .

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Because we want to -- you want to have the -- it's not just the average, it's the percent of people above with reduction. Remember, there's always variability in the drug and how much you on how much you reduce it. And so we want to make sure that everyone gets benefit.

Umer Raffat - EVERCORE ISI - Analyst

And then my last one on this is really I remember -- some of the initial studies on PCSK9, one of the feedback was that if you enroll a population which is very well controlled and their LDL levels are already at 70 or 80, you're not going to get a massive hazard ratio on a (inaudible) and I think that's kind of the framework you guys are using for the most part in our CVOT as well in the outcomes trial. I guess, how did you guys think through that, having patients above 100 LDL could have resulted in a very different hazard ratio, presumably.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

No, I don't -- so the hazard ratio is a combination of -- first of all, there's no -- there's no such thing as too low of a cholesterol levels. So it will get benefit across the range. Hazard ratios for CVOTs are also dependent on time. And I think if you looked at some of the other PCSK9 inhibitors, they were truncated a little early, and that's why you had a hazard ratios that maybe were not at the numbers that you'd want to see. We've done a lot of CVOTs.

So we've modeled this as that the hazard ratio is good.

Umer Raffat - EVERCORE ISI - Analyst

And when should we get a readout on this?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

And do we say? I think is 2028. 2029, yes. It's still enrolling. So.

Umer Raffat - EVERCORE ISI - Analyst

Peter, I don't know if you guys have said much on this. Do you guys expect this to be a meaningful launch before the CDLT.

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

Yes. We will plan to launch based on the biomarker data.

Umer Raffat - EVERCORE ISI - Analyst

And to the extent you putting so much investment on this, would it make sense to have whatever you want to do in obesity also be in the marketplace around the same time? Is that how you guys are thinking about the preferences of whatever molecule you guys are looking into for obesity.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Well, I mean, I think we've got a bunch of different -- each molecule stands for its itself. So we're looking at -- for this particular molecule, we're going to -- the data for enlicitide we'll have the cardiovascular outcome data we said in 28, but we'll have the cholesterol lowering by the end of next year, beginning of 26. And so that's when we'll start that. MK-6024, which is our MASH compound, that's an injectable.

-- you'll have Phase IIb results who, I think late next year, and then Phase 3 will start after that should the data warrant it. great enthusiasm with that drug is our enrollment rates have been through the roof. And so we're -- I think that that's where our current metabolic play is, we'll see what we -- what happens with either business development or through our own labs in terms of other mechanisms of action.

Umer Raffat - EVERCORE ISI - Analyst

It sounds like you guys may just go with an in-house approach because all .Previously, it sounded like you guys are certainly looking at an external asset for obesity..

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Well, I think -- so just to step back, the question in obesity is what can be added to what's already out there in the [armamentarium]. If you think about all the different assets that are there in obesity, they're injectables, there are all kinds of different orals. A lot of companies have gotten into it. And so the question is, what we've been looking at as we saw the field and we saw the wonderful work that's being done with injectables and now some of the orals you have to -- it's not good enough just to have something. You have to have something that's differentiated and that's relevant and that you can put into a suite of assets that make a difference for patients.

And so we've been pretty disciplined about what we want to see, whether it's from our own internal folks or from the outside. But it has to be something that's not just another oral.

Umer Raffat - EVERCORE ISI - Analyst

Sir differentiated at this point. So maybe let me just set the parameters weight loss, we're hitting 25% with injectables right now. Or the other way of doing it is orals, which is mid-teens or a third parameter as you go into the muscle. Amylin perhaps could fall into that. We need to see confirmation of that. Are there other parameters you see?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

So I think those are important parameters. There's tolerability, which is an important parameter. There's orthogonal. There's the question about whether or not if you had orthogonal mechanisms that are or not yet. -- been part of the -- all the names, the drug targets you've talked about whether or not there's something that with lower doses of each, you can get something that's a little bit more effective .

Umer Raffat - EVERCORE ISI - Analyst

But if you goal, don't you need a background regardless to put the orthogonal on to.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Maybe, we'll have to see. I mean, I think that the point is, though, is that just having another equivalent of an oral agent when there is -- you can count a whole bunch of stuff that's, of course, some of them are in Phase 1 or 1b/2 know for a fact that they're going to get to the end, but these are serious companies with serious drugs. Just having another one of those and starting is not necessarily the case. So you have to have a combination

-- some sort of combination approach maybe or some sort of unique feature that will help out with things like tolerability, as you mentioned, how much weight loss, any effects on drug-drug interactions?

Can you eliminate the problem with muscle wasting and it goes -- you can go on and on about different things as the drugs evolve. So the way I look at the obesity field is that it's there's this initial gold rush. And then the question is what really will stay the course in the long term? And that remains to be -- that's going to be a more complicated question.

We didn't get into the initial gold rush it wasn't our thing. We have so many other things in the pipeline. So now it's -- now I'm talking about in general. And so the question about obesity is a question about what will -- what is a Merck product. What is something that we can do that we can show differentiation -- it's not so simple and has to be a very thoughtful approach.

And so we haven't run around and just bought things willy-nilly.

Peter Dannenbaum - Merck & Co Inc - Senior Vice President, Investor Relations

And just to be clear, Umer, that messaging has been consistent. So there's no intentional change. We have internal programs, preclinical, and we continue to look at BD as a potential opportunity.

Umer Raffat - EVERCORE ISI - Analyst

Could you have anything in the clinic in obesity by next year? .

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I don't know see. I mean I just have to -- it all depends. Again, we have a very rigorous gates for everything that we do because we've got like 0 -- depending on how you count it. molecules. I've got -- I'm very confident in the amount of molecules that I have in the clinic so far.

I think Merck has got a tremendous pipeline as it is, not including obesity. And I'm -- and so I'm really excited about the potential opportunities there, but I'm not -- I'm not wholly dependent on it. I'm going to use the same kind of strong stage gating that I do for any other -- anything else, and we're not going to run to the field like as if oh my god, we have to be there.

Umer Raffat - EVERCORE ISI - Analyst

Maybe a quick second on vaccines. You guys talked about a program called V118. So this is not 117, it's 118. Is this a potential 30-plus valent that could do infants and adults for pneumococcal vaccines?.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

The issue -- so what are the issues with pneumococcal vaccines that you want to address? You want to address maximize either pediatric or adult. And you also want to make sure that ironically enough, as you add types and if your immunogenicity goes down, it's actually what's the residual of the types that have been in the vaccine already where the efficacy isn't 100%, that's actually where most of the diseases. If you look at pediatrics and adult Serotype 3 is a much bigger disease burden, although in the vaccines compared with all of these number 29, 30, 31, 32.

So what we're looking at with V118 is we want to make sure that we have robust vaccine that could address with high immunogenicity, not just new types and sort of say, oh, look, 30 is better than 25 is better than 20, but actually speaks to what we anticipate the verge of disease will be reduced. And that could be in adults or in children, when one has to, we'll see where we go.

Umer Raffat - EVERCORE ISI - Analyst

So we shouldn't hold our breath on this being a 30-plus valid, for example.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I didn't say that.

Umer Raffat - EVERCORE ISI - Analyst

Okay. And the next-gen GARDASIL that's moving forward, is that a 14 balance?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's going to be to increase the number of types, particularly those patients who have from more diverse settings where we haven't been able to hit with GARDASIL 9. The idea with this vaccine is to fundamentally change cervical cancer screening. So it's going to try to maximize the number of -- we haven't announced the number of types, but my goal with that is to have sufficient coverage that patients won't -- that women won't have to undergo routine cervical cancer screening if they're fully vaccinated.

Umer Raffat - EVERCORE ISI - Analyst

Got it. One last, sorry. On pneumoconal vaccine, I totally forgot -- there's some evidence in the patent domain would suggest Pfizer for their really broad spectrum. So the 25-valent and the 30-plus-valent, they're using a second carrier protein to fix serotype 3 I know you talked about serotype 3 there as well. So on the back of my head, I was wondering, is Merck now also considering two (inaudible) proteins?

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Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

This has been -- so Serotype 3 has been a real Achilles heel for people. And by the way, and also 19F and others. So it's not like the only one. And so I think that it's important to look at other approaches at a variety of approaches. It could be adjuvant.

It could be carrier protein. And you see the literature, and you see some of the other vaccines that are out there coming with different approaches to either conjugation or to sort of the system in which the vaccine is manufactured. So we have a variety of approaches. There are many subtypes of V118 and we'll figure out the right one from -- based on data.

Umer Raffat - EVERCORE ISI - Analyst

This one is entering clinic when 118.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I think we phase is one, yes.

Umer Raffat - EVERCORE ISI - Analyst

It is in Phase 1, there's multiple formulations.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

There'll be -- we're going to have a series of -- of different 118s.

Umer Raffat - EVERCORE ISI - Analyst

There will be a series of formulations. I see. I see. Okay. It makes a ton of sense.

So last one for me in the last minute, it's -- I should know more about this, and I don't actually. MK-8527 in HIV -- this is a readout coming up pretty soon. This is for PrEP with a single pill once a month.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Absolutely.

Umer Raffat - EVERCORE ISI - Analyst

But the dose is really high.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

No.

Umer Raffat - EVERCORE ISI - Analyst

What is it?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

The dose is really low. It's a I forget how much is -- it's much lower.

Umer Raffat - EVERCORE ISI - Analyst

Wasn't it like 100 milligrams or something from what I recall.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's going to be a once-monthly pill small tablet regimen.

Umer Raffat - EVERCORE ISI - Analyst

Okay. I guess how should we think about leukopenia risks or any CD4 issues that could happen by pushing it that far?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

We'll find out soon enough. But 8527 is not like, it's in the same family as islatravir, but it's a different molecule, different threshold. So I'll tell you.

Umer Raffat - EVERCORE ISI - Analyst

Is it same scaffold.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes. But it's a scaffold, different molecule. But I think it's -- this is going to be a big deal.

Umer Raffat - EVERCORE ISI - Analyst

Should we not worry about CD4 drop then.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

You should always worry about safety until you see the data. So in my mind, the way I look at it is if you could have a single pill once a month, that's not a horse pill, the people can take that is the sine qua non of what PrEP should be. It just ask the Gates Foundation and those guys, they always are asking about this sort of stuff. We're very excited about this molecule.

Umer Raffat - EVERCORE ISI - Analyst

I feel like no one ever talked to you guys about this program, and this is a readout coming up imminently, right?

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

I was formerly head of ID vaccines at Merck, and I never know why people never talked. They always talk about that Gardasil issue in China. You're talking about 8527.

Umer Raffat - EVERCORE ISI - Analyst

8527. So this is being developed only for PrEP, not for treatment.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Yes. Just for PrEP.

Umer Raffat - EVERCORE ISI - Analyst

And why not for treatment too.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Because we've got 8591 for that. We've got Islatravir for that. No need.

Umer Raffat - EVERCORE ISI - Analyst

But isn't Islatravir, like in my mind, it's like clouded pass now with the CD4 and FDA might require monitoring or just give you a black box.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Just wait until you -- The data are coming. Just -- we worked on the dose, the q day regimen data are reading out post haste. And so you'll see it soon enough, and I'm very confident that Merck is back in HIV and I mean it. Yes.

Umer Raffat - EVERCORE ISI - Analyst

Just wait is my note on Islatravir. Okay, got it. But not with the 8527 on both sides.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

8527 is for PrEP.

Umer Raffat - EVERCORE ISI - Analyst

Okay. Got it.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It's just that they've got the right profile for PrEP. It will be its own regimen. Islatravir is incredibly well characterized, Yes, including its high-end toxicity if you give too much of it. But we already have it in q week and the q day. There's no reason to go with 8527 for that particular setting.

We'll just have it for PrEP.

Umer Raffat - EVERCORE ISI - Analyst

So last one to finish off on virology. Molnupiravir, any additional indications are you guys putting in more investments because that looks like a promising molecule.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Well, We are doing another efficacy study in COVID. I think it's really important to -- this -- this is one of those drugs that I cannot believe that we had such a weird press about. Because if you go to Asia, in Japan and in Australia and so on. That's been the drug that they've used for COVID, and it's been a smashing success. And so I don't know why -- well, there's been all kinds of weird social media sort of stuff and whatever in the US.

We've looked at a variety of different indications, other viruses with challenge studies. Data are mixed, but we are investing in another COVID study because we really believe in this drug for COVID.

Umer Raffat - EVERCORE ISI - Analyst

Is that -- and there's a viral challenge studies in other indications. Is that published anywhere? .

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

No, it's coming. And so we're.

Umer Raffat - EVERCORE ISI - Analyst

Because mechanistically, I thought it lends itself to (inaudible).

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

It could you have to -- that's why you always do the study just because.

Umer Raffat - EVERCORE ISI - Analyst

And the viral challenge was done with the right dose of the.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

Right. So we have a different viruses and those data will come in due course. But I think the biggest -- the place where we are sure that it's efficacious, is in COVID and where -- but the regulatory authorities in the United States, FDA has asked for another study, and that study will commence.

Umer Raffat - EVERCORE ISI - Analyst

Got it.

Eliav Barr - Merck & Co Inc - Senior Vice President, Head of Global Clinical Development and Chief Medical Officer

And I'm confident that it's going to be positive.

Umer Raffat - EVERCORE ISI - Analyst

Excellent. Anything in the audience just before we wrap up? Alright, thank you, guys.

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